

ICS Activation Checklist

— Medical Emergency

Printable version of the on-screen checklist for the Medical Emergency scenario.

Time	Step	Actions	Lead / support	Confirmations
0–1 min	Step 1 — Check safety	- Confirm scene safety before approaching; stop the cause of injury (vehicle off, power cut, wildlife clear). - Assess number of victims, hazards, level of consciousness; assign a team member to crowd control. - Do not move the victim unless in imminent danger; clear non-essential people from the area. - Notify IC/OC: "Code Medical — [Location] — [number of patients]."	1st Responder / Nearby Team / Security	☐ Confirmed safety ☐ Location, victims, hazards, consciousness confirmed ☐ Alarm raised, IC informed
1–3 min	Step 2 — Activate ICS	- IC activates ICT/ERT and assigns OC, LC, EC, Medic, Responders; EC logs activation time. - IC notifies Head Office and activates external medical support; EC maintains the medical log. - OC assigns Responders to secure the area; LC assists with transfers and equipment.	IC / EC / OC / LC	☐ ICT/ERT activated, roles assigned ☐ EC log started ☐ HO notified · Medical support contacted (AMREF) ☐ Safety perimeter set · Transport & equipment prepared
3–5 min	Step 3 — Medical assessment	- Medic assesses Airway/Breathing/Circulation and initiates first aid. - Prepare for transport to the nearest medical facility; OC secures and crowd-controls the area. - Check other guests for shock; cover, warm, raise legs, reassure — no alcohol.	Medic / OC / Responders	☐ Patient stabilised ☐ Transport arranged if required ☐ Area secured · Guests checked for shock
5–10 min	Step 4 — Evacuation prep	- LC prepares the vehicle; EC contacts AMREF / Flying Doctors / Arusha Medivac if instructed by IC. - Medic stabilises the patient for movement; IC authorises road or air evacuation. - Gather information for the AMREF checklist and Medical Incident Report.	LC / EC / Medic / IC	☐ Transport prepared · Evac approved ☐ Medevac provider contacted if needed ☐ Medical report completed for handover
10–15 min	Step 5 — External coordination	- IC maintains command and communication with Head Office and authorities. - EC provides ETA to Head Office and the medevac service. External comms normally go via the IC, except in a life-threatening emergency: contact services directly, then notify the IC.	IC / EC	☐ Head Office updated
15–30 min	Step 6 — Evacuation / transfer	- Patient moved safely; OC clears the path, LC guides to the landing zone or gate. - Responders assist lifting; arrange alternative transport for unaffected guests; Medic monitors vitals. - Night landings: LC prepares airstrip lighting — vehicle headlights into wind, lamps along the runway.	OC / LC / Medic	☐ Patient transferred · other transfers arranged ☐ Night evac airstrip lights prepared if needed
30–60 min	Step 7 — Handover	- Medic gives a verbal and written summary to ambulance/flight crew. - EC records the handover name and time; IC confirms safe handover and updates Head Office.	Medic / EC / IC	☐ Transfer logged · handover time recorded ☐ Head Office updated
Within 24 hrs	Step 8 — Debrief & reporting	- IC leads a 15-minute debrief within 2 hours of All Clear. - EC completes the Medical Incident Report and submits to Head Office within 24 hours with IC approval. - Medic restocks supplies and reviews equipment condition.	IC / EC / Medic / OC / LC	☐ Report submitted · debrief done ☐ Supplies restocked